

APALIFE 10 mg Tablet**APALIFE 15 mg Tablet****ANTIPSYCHOTIC**

Each film coated tablet contains:

Aripiprazole 10 mg**Aripiprazole 15 mg**❖ **PRODUCT DESCRIPTION:****10 mg:**

Pink, modified rectangle, biconvex, film coated tablet with engraved 10 on one side and plain on the other

15 mg:

Yellow, round, biconvex, film coated tablet with engraved 15 on one side and plain on the other

❖ **MECHANISM OF ACTION:****Pharmacology**

It has been proposed that Aripiprazole's efficacy in schizophrenia and Bipolar I Disorder is mediated through a combination of partial agonism at dopamine D₂ and serotonin 5HT_{1a} receptors and antagonism of serotonin 5HT_{2a} receptors. Aripiprazole exhibited antagonist properties in animal models of dopaminergic hyperactivity and agonist properties in animal models of dopaminergic hypoactivity. Aripiprazole exhibited high binding affinity *in vitro* for dopamine D₂ and D₃, serotonin 5HT_{1a} and 5HT_{2a} receptors and moderate affinity for dopamine D₄, serotonin 5HT_{2c} and 5HT₇, alpha-1 adrenergic and histamine H₁ receptors. Aripiprazole also exhibited moderate binding affinity for the serotonin reuptake site and no appreciable affinity for muscarinic receptors. Interaction with receptors other than dopamine and serotonin subtypes may explain some of the other clinical effects of Aripiprazole.

Pharmacokinetics**Absorption**

Aripiprazole is well absorbed, with peak plasma concentrations occurring within 3-5 hours after dosing. Aripiprazole undergoes minimal pre-systemic metabolism. The absolute oral bioavailability of the tablet formulation is 87%. There is no effect of a high fat meal on the pharmacokinetics of Aripiprazole.

Distribution

Aripiprazole is widely distributed throughout the body with an apparent volume of distribution of 4.9 l/kg, indicating extensive extravascular distribution. At therapeutic concentrations, Aripiprazole and dehydro-Aripiprazole are greater than 99% bound to serum proteins, binding primarily to albumin.

Biotransformation

Aripiprazole is extensively metabolized by the liver primarily by three biotransformation pathways: dehydrogenation, hydroxylation, and N-dealkylation. Based on *in vitro* studies, CYP3A4 and CYP2D6 enzymes are responsible for dehydrogenation and hydroxylation of Aripiprazole, and N-dealkylation is catalyzed by CYP3A4. Aripiprazole is the predominant medicinal product moiety in systemic circulation. At steady state, dehydro-Aripiprazole, the active metabolite, represents about 40% of Aripiprazole AUC in plasma.

Elimination

The mean elimination half-lives for Aripiprazole are approximately 75 hours in extensive metabolizers of CYP2D6 and approximately 146 hours in poor metabolizers of CYP2D6. The total body clearance of Aripiprazole is 0.7 mL/min/kg, which is primarily hepatic. Following a single oral dose of [¹⁴C]-labelled Aripiprazole, approximately 27% of the administered radioactivity was recovered in the urine and approximately 60% in the feces. Less than 1% of unchanged Aripiprazole was excreted in the urine and approximately 18% was recovered unchanged in the feces.

❖ **INDICATION:**

Aripiprazole is indicated for the treatment of acute episodes of schizophrenia and for maintenance of clinical improvement during continuation therapy. Aripiprazole is indicated for the treatment of acute manic episodes associated with Bipolar I Disorder.

❖ **DOSAGE AND ADMINISTRATION:****Oral****Adults**

Schizophrenia - The recommended starting dose for Aripiprazole is 10 or 15 mg/day administered on a once-a-day schedule without regard to meals. The maintenance dose for Aripiprazole is 15 mg/day. Doses in the range of 10 to 30 mg/day have been established as effective in clinical trials.

Bipolar Mania - Aripiprazole should be administered on a once-a-day schedule without regard to meals, generally starting at a dose of 15 or 30 mg/day. Dose adjustments, if indicated, should occur at intervals of not less than 24 hours. Antimanic efficacy (3-12 weeks) was demonstrated in a dose range of 15 to 30 mg/day in clinical trials. The safety of doses above 30 mg/day has not been evaluated in clinical trials.

Special populations

Renal impairment - No dosage adjustment is required in patients with renal impairment.

Hepatic impairment - No dosage adjustment is required for patients with hepatic impairment (Child-Pugh Class A, B, or C).

Pediatric - The safety and efficacy of Aripiprazole in patients under 18 years of age have not been established.

Elderly - No dosage adjustment is required for patients 65 years of age. However, experience with this patient population is limited.

Gender - No dosage adjustment is required for female patients relative to male patients.

Patients taking medications metabolized by CYP2D6 or 3A4 – Dosage adjustment for patients taking Aripiprazole concomitantly with potent CYP3A4 or CYP2D6 inhibitors: When concomitant administration of a potent CYP3A4 or CYP2D6 inhibitor with Aripiprazole occurs, the Aripiprazole dose should be reduced to one-half of the usual dose. When the CYP3A4 or CYP2D6 inhibitor is withdrawn from the combination therapy, the Aripiprazole dose should then be increased.

Dosage adjustment for patients taking potent CYP3A4 inducers: When a potent CYP3A4 inducer is added to Aripiprazole therapy, the Aripiprazole dose should be doubled. Additional dose increases of Aripiprazole should be based on clinical evaluation. When the CYP3A4 inducer is withdrawn from the combination therapy, the Aripiprazole dose should be reduced. Consideration should be given to reducing the daily dose in individual patients who are on multiple concomitant medications that inhibit CYP3A4 and CYP2D6 enzymes.

Smoking status - No dosage adjustment is required for smoking patients relative to non-smoking patients.

❖ **PRECAUTION:**

During antipsychotic treatment, improvement in the patient's clinical condition may take several days to some weeks. Patients should be closely monitored throughout this period.

- Suicidality, Cardiovascular disorders, Conduction abnormalities, Tardive dyskinesia, Other extrapyramidal symptoms, Neuroleptic Malignant Syndrome (NMS), Seizure, Elderly patients with dementia-related psychosis (Increased mortality, Cerebrovascular adverse reactions), Hyperglycemia and diabetes mellitus, Hypersensitivity, Weight gain, Dysphagia, Pathological gambling, Lactose, Phenylketonurics, Intolerance, Patients with ADHD comorbidity.

Product	Code No.	Dimension	Packaging Type	Thickness
APALIFE	IAMY 0051	W 16.5 x L 27 cm.	Wood Free Paper (กระดาษปลอดไม้)	60 g (0.08 mm.)
Designed by: (PDM, ASPD, PDCM, PDCA, PDQM, PDDA) Checked by: (PDM/ASPD) Approved by: (MD, (Only Pattern and Color)) IRA: (Date) QCC-PMT: (Date) CUSTOMER/SALES DEPT.: (Date)				
PM Specification	Approved by: (PDM, ASPD, PDCM, PDCA, PDQM, PDDA) Checked by: (PDM/ASPD) Approved by: (MD, (Only Pattern and Color)) IRA: (Date) QCC-PMT: (Date) CUSTOMER/SALES DEPT.: (Date)			

❖ PREGNANCY AND LACTATION:

Pregnancy

Neonates exposed to antipsychotic drugs during the third trimester of pregnancy are at risk for extrapyramidal and/or withdrawal symptoms following delivery. There have been reports of agitation, hypertonia, hypotonia, tremor, somnolence, respiratory distress, and feeding disorder in these neonates. These complications have varied in severity; while in some cases symptoms have been self-limited, in other cases neonates have required intensive care unit support and prolonged hospitalization. APALIFE should be used during pregnancy only if the potential benefit justifies the potential risk to the fetus.

Breast-feeding

Aripiprazole is excreted in human breast milk. Patients should be advised not to breast feed if they are taking Aripiprazole.

❖ WARNING:

Hyperglycemia and Diabetes Mellitus

Hyperglycemia in some cases extreme and associated with ketoacidosis or hyperosmolar coma or death, has been reported in patients treated with atypical antipsychotics. Assessment of the relationship between atypical antipsychotics use and glucose abnormalities is complicated by the possibility of an increased background risk of diabetes mellitus in patients with schizophrenia and the increasing incidence of diabetes mellitus in general population. Given this confounders, the relationship between atypical antipsychotic use and hyperglycemia-related adverse events is not completely understood. However, epidemiological studies suggest an increased risk of treatment-emergent hyperglycemia-related adverse events in patients treated with atypical antipsychotics. Precise risk estimates for hyperglycemia-related adverse events in patients treated with atypical antipsychotics are not available.

Patients with an established diagnosis of diabetes mellitus who are started on atypical antipsychotics should be monitored regularly for worsening of glucose control. Patients with risk factors for diabetes mellitus (e.g. obesity, family history of diabetes) who are starting treatment with atypical antipsychotics should undergo fasting blood glucose testing at the beginning of treatment and periodically during treatment. Any patient treated with atypical antipsychotics should be monitored for symptoms of hyperglycemia including polydipsia, polyuria, polyphagia, and weakness. Patients who develop symptoms of hyperglycemia during treatment with atypical antipsychotics should undergo fasting blood glucose testing. In some cases, hyperglycemia has resolved when the atypical antipsychotic was discontinued; however, some patients required continuation of anti-diabetic treatment despite discontinuation of the suspect drug.

Pathological gambling and impulse-control problems

Patients can experience increased urges, particularly for gambling, and the inability to control these urges while taking Aripiprazole. Other urges reported include: increased sexual urges, compulsive shopping, binge or compulsive eating, and other impulsive and compulsive behaviors.

It is important for prescribers to ask patients or their caregivers specifically about the development of new or increased gambling urges, or other urges, while being treated with Aripiprazole. It should be noted that impulse-control symptoms can be associated with underlying disorder; however, in some cases urges were reported to have stopped when the dose was reduced or the medication was discontinued. Patients who are at higher risk for impulse-control problems (e.g. personal or family history of obsessive-compulsive disorder, impulse-control disorder, bipolar disorder, impulsive personality, alcoholism, drug abuse or other addictive behaviors) would require monitoring for new or worsening of uncontrollable urges. Impulse-control problems may results in harm to the patient and others if not recognized. Consider dose reduction or stopping the medication if a patient develops such urges while taking Aripiprazole.

❖ SIDE EFFECT:

Headache, fatigue, tremor, anxiety, insomnia, increased appetite, N/V, somnolence, constipation, akathisia, extrapyramidal disorder, nasopharyngitis, dizziness, restlessness, weight increase.

Psychiatric disorders

Pathological gambling, hypersexuality, impulse-control problems (See warning section.)

❖ CONTRAINDICATION:

Hypersensitivity to the active substance or to any of the excipients

❖ DRUG INTERACTION*:

Caution with other centrally acting drugs. Avoid with alcohol. May potentiate the effect of certain antihypertensive agents. CYP3A4 inducers (e.g. Carbamazepine) may increase clearance and lower blood levels. CYP3A4 inhibitors (e.g. Ketoconazole, Itraconazole) or CYP2D6 inhibitors (e.g. Quinidine, Fluoxetine, Paroxetine) may inhibit elimination and increase blood levels.

❖ OVERDOSE AND TREATMENT:

Signs and symptoms

The potentially medically important signs and symptoms observed included lethargy, increased blood pressure, somnolence, tachycardia, nausea, vomiting and diarrhea. In addition, reports of accidental overdose with Aripiprazole alone (up to 195 mg) in children have been received with no fatalities. The potentially medically serious signs and symptoms reported included somnolence, transient loss of consciousness and extrapyramidal symptoms.

Management of overdose

Management of overdose should concentrate on supportive therapy, maintaining an adequate airway, oxygenation and ventilation, and management of symptoms. The possibility of multiple medicinal product involvement should be considered. Therefore cardiovascular monitoring should be started immediately and should include continuous electrocardiographic monitoring to detect possible arrhythmias. Following any confirmed or suspected overdose with Aripiprazole, close medical supervision and monitoring should continue until the patient recovers.

Activated charcoal (50 g), administered one hour after Aripiprazole, decreased Aripiprazole C_{max} by about 41% and AUC by about 51%, suggesting that charcoal may be effective in the treatment of overdose.

Hemodialysis

Although there is no information on the effect of hemodialysis in treating an overdose with Aripiprazole, hemodialysis is unlikely to be useful in overdose management since Aripiprazole is highly bound to plasma proteins.

❖ STORAGE:

Store at temperature of not more than 30°C.

❖ DOSAGE FORM AND PACKAGING AVAILABLE:

10 mg Tablet, Blister 3x10's
15 mg Tablet, Blister 3x10's

❖ DATE OF REVISION:

May 10, 2023

Manufactured by:
UNISON LABORATORIES CO., LTD.
39 Moo 4, Klong Udomcholjorn, Muang Chachoengsao,
Chachoengsao 24000 Thailand

Product	Code No.	Dimension	Packaging Type	Thickness
APALIFE	IAMY 0051	W 16.5 x L 27 cm.	Wood Free Paper (กระดาษปลอดกาว)	60 g (0.08 mm.)
 PM Specification Designed by: (PDM, ASPD, PDCM, PDCA, PDCM, PDCM) Checked by: (PDM/ASPD) Approved by: (MD, Only Patient and Coder) IRA: (Date) QCC-PM: (Date) CUSTOMER/SALES DEPT.: (Date) PLC: (Date) Approved by: (Date)				